



BINOVEN-ER 37.5
(Venlafaxine Extended Release Capsules 37.5 mg)
BINOVEN-ER 75
(Venlafaxine Extended Release Capsules 75 mg)
BINOVEN-ER 150
(Venlafaxine Extended Release Capsules 150 mg)

Description and Composition:

BINOVEN-ER 37.5(Venlafaxine Extended Release Capsules 37.5 mg):

White to off white spherical to oval pellets filled in Empty hard gelatin capsule shell (size '3') of Opaque Grey color cap and Opaque Peach color body imprinted with "E" on cap and "73" on the body with edible black ink.

Each extended- release capsule contains Venlafaxine hydrochloride Ph. Eur. equivalent to 37.5 mg Venlafaxine.

BINOVEN-ER 75 (Venlafaxine Extended Release Capsules 75 mg):

White to off white spherical to oval pellets filled in Empty hard gelatin capsule shell (size '1') of Opaque Peach color cap and Opaque Peach color body imprinted with "E" on cap and "74" on the body with edible black ink.

Each extended- release capsule contains Venlafaxine hydrochloride Ph. Eur. equivalent to 75 mg Venlafaxine.

BINOVEN-ER 150 (Venlafaxine Extended Release Capsules 150 mg)

White to off white spherical to oval pellets filled in Empty hard gelatin capsule shell (size '0') of Opaque Dark Orange color cap and Opaque Dark Orange color body imprinted with "E" on cap and "89" on the body with edible black ink.

Each extended- release capsule contains Venlafaxine hydrochloride Ph. Eur. equivalent to 150 mg Venlafaxine.

Pharmacodynamics

Antidepressants

The mechanism of venlafaxine's antidepressant action in humans is believed to be associated with its potentiation of neurotransmitter activity in the central nervous system. Venlafaxine and its major metabolite, O-desmethyl venlafaxine (ODV), are inhibitors of serotonin and noradrenaline reuptake. Venlafaxine also weakly inhibits dopamine uptake. Venlafaxine and its active metabolite reduce β -adrenergic responsiveness after both acute (single dose) and chronic administration. Venlafaxine and ODV are very similar with respect to their overall action on neurotransmitter reuptake and receptor binding. Venlafaxine has virtually no affinity for rat brain muscarinic, cholinergic, H1- histaminergic or α 1-adrenergic receptors in vitro. Pharmacological activity at these receptors may be related to various side effects seen with other antidepressant medicinal products, such as anticholinergic, sedative and cardiovascular side effects. Venlafaxine does not possess monoamine oxidase (MAO) inhibitory activity.

Pharmacokinetics

Venlafaxine is extensively metabolised, primarily to the active metabolite, O- desmethylvenlafaxine (ODV). Venlafaxine and ODV exhibit linear kinetics over the dose range of 75 mg to 450 mg/day.

Absorption

At least 92% of venlafaxine is absorbed following single oral doses of immediate-release venlafaxine. Absolute bioavailability is 40% to 45% due to presystemic metabolism. After immediate-release venlafaxine administration, the peak plasma concentrations of venlafaxine and ODV occur in 2 and 3 hours, respectively. Following the administration of venlafaxine prolonged-release capsules, peak plasma concentrations of venlafaxine and ODV are attained within 5.5 hours and 9 hours, respectively. When equal daily doses of venlafaxine are administered as either an immediate-release tablet or prolonged-release capsule, the prolonged-release capsule provides a slower rate of absorption, but the same extent of absorption compared with the immediate-release tablet. Food does not affect the Bioavailability of Venlafaxine and ODV.

Distribution

Venlafaxine and ODV are minimally bound at therapeutic concentrations to human plasma proteins (27% and 30%, respectively). The volume of distribution for venlafaxine at steady-state is 4.4 $\pm$ 1.6 L/kg following intravenous administration.

Metabolism

Venlafaxine undergoes extensive hepatic metabolism that Venlafaxine is bio transformed to its major active metabolite, ODV, by CYP2D6. Venlafaxine is metabolised to a minor, less active metabolite, N-desmethylvenlafaxine, by CYP3A4. Venlafaxine is a weak inhibitor of CYP2D6. Venlafaxine did not inhibit CYP1A2, CYP2C9, or CYP3A4.

Elimination

Venlafaxine and its metabolites are excreted primarily through the kidneys. Approximately 87% of a venlafaxine dose is recovered in the urine within 48 hours as either unchanged venlafaxine (5%), unconjugated ODV (29%), conjugated ODV (26%), or other minor inactive metabolites (27%).

Special populations

Age and gender

Subject age and gender do not significantly affect the pharmacokinetics of venlafaxine and ODV.

CYP2D6 extensive/poor metabolisers

Plasma concentrations of venlafaxine are higher in CYP2D6 poor metabolisers than extensive metabolisers. Because the total exposure (AUC) of venlafaxine and ODV is similar in poor and extensive metabolisers, there is no need for different venlafaxine dosing regimens for these two groups.

Patients with hepatic impairment

Venlafaxine and ODV half-lives were prolonged compared to normal subjects. The oral clearance of both venlafaxine and ODV was reduced.

Patients with renal impairment

Venlafaxine elimination half-life was prolonged by about 180% and clearance reduced by about 57% compared to normal subjects, while ODV elimination half-life was prolonged by about 142% and clearance reduced by about 56%. Dosage adjustment is necessary in patients with severe renal impairment and in patients that require haemodialysis.

Indications

Treatment of depression, including depression associated with anxiety, in hospitalized patients. Treatment of anxiety or generalized anxiety disorder (GAD), including long-term treatment. The effectiveness of Venlafaxine Hydrochloride Extended Release in long- term use for GAD has been evaluated for up to 6 months in controlled clinical trials.

For prevention of relapse of an episode of depression or for prevention of the recurrent of new depressive episodes.

BINOVEN-ER is indicated for the treatment of Social Anxiety Disorder, also known as social phobia, as defined in DSM-IV.

Social Anxiety Disorder (DSM-IV) is characterized by a marked and persistent fear of one or more social or performance situations in which the person is exposed to unfamiliar people or to possible scrutiny by others. Exposure to the feared situation almost invariably provoke anxiety, which may approached the intensity of a panic attack. The feared situation are avoided or endured with intense anxiety or distress. The avoidance, anxious anticipation or distress in the feared situation(s) interferes significantly with the person's normal routine, occupational or academic functioning or social activities or relationship, or there is marked distress about having the phobias. Lesser degrees of performance anxiety or shyness generally do not require psychopharmacological treatment.

Panic disorder, including prevention of relapse.

Recommended dose

General Recommendation

It is recommended that venlafaxine extended-release capsules be taken with food, at approximately the same time each day. Capsules must be swallowed whole with fluid and not divided, crushed, chewed, or dissolved, or it may be administered by carefully opening the capsule and sprinkling the entire contents on a spoonful of applesauce. This drug/food mixture should be swallowed immediately without chewing and followed with a glass of water to ensure complete swallowing of the pellets.

With the exception of patients with SAD, patients not responding to the 75 mg/day dose may benefit from dose increases in increments of up to 75 mg/day to a maximum of 225 mg/day.

Extended-release venlafaxine dosage increases can be made at intervals of 2 weeks or more, but not less than 4 days.

Patients treated with venlafaxine immediate-release tablets may be switched to venlafaxine extended-release capsules at the nearest equivalent daily dosage. For example, venlafaxine immediate-release tablets 37.5 mg twice daily may be switched to venlafaxine extended-release capsules 75 mg once daily. Individual dosage adjustments may be necessary.

Major Depressive Disorder

The recommended starting dose of extended-release venlafaxine is 75 mg given once daily. Patients not responding to the initial 75 mg/day dose may benefit from dose increases to a maximum 225 mg/day.

The venlafaxine dose can be titrated up to 225 mg/day in moderately depressed patients and 375 mg/day for severely depressed patients.

Generalized Anxiety Disorder

The recommended starting dose for extended-release venlafaxine is 75 mg per day given once daily. Patients not responding to the initial 75 mg/day dose may benefit from dose increases to a maximum of 225 mg/day.

Social Anxiety Disorder

The recommended starting dose for extended-release venlafaxine is 75 mg/day, given once daily. There is no evidence that higher doses confer any additional benefit.

Panic Disorder

It is recommended that a dose of 37.5 mg/day of venlafaxine be used for 7 days. Dosage should then be increased to 75 mg/day. Patients not responding to the 75 mg/day dose may benefit from dose increases to a maximum of 225 mg/day.

Discontinuing Venlafaxine

Dose tapering is recommended whenever possible when discontinuing venlafaxine therapy. Tapering was achieved by reducing the daily dose by 75 mg at 1-week intervals. The period required for tapering may depend on the dose, duration of therapy, and the individual patient.

Use in Patients with Renal Impairment

The total daily dose of venlafaxine should be reduced by 25% to 50% for patients with renal impairment with a glomerular filtration rate (GFR) of 10 to 70 mL/min.

The total daily dose of venlafaxine should be reduced by 50% in hemodialysis patients.

Because of individual variability in clearance in these patients, individualization of dosage may be desirable.

Use in Patients with Hepatic Impairment

The total daily dose of venlafaxine should be reduced by 50% in patients with mild to moderate hepatic impairment. Reductions of more than 50% may be appropriate for some patients.



Because of individual variability in clearance in these patients, individualization of dosage may be desirable.

Use in Children and Adolescent

There is insufficient experience with the use of venlafaxine in patients less than 18 years of age.

Use in Elderly Patients

No specific dosage adjustments of venlafaxine are recommended based on patient age.

Mode of administration

Oral route of administration.

Contraindication

Hypersensitivity to the active substance or to any of the excipients.

Concomitant treatment with irreversible monoamine oxidase inhibitors (MAOIs) is contraindicated due to the risk of serotonin syndrome with symptoms such as agitation, tremor and hyperthermia. Venlafaxine must not be initiated for at least 14 days after discontinuation of treatment with an irreversible MAOI. Venlafaxine must be discontinued for at least 7 days before starting treatment with an irreversible MAOI.

Warning and precautions

Suicide/suicidal thoughts or clinical worsening

Depression is associated with an increased risk of suicidal thoughts, self-harm and suicide (suicide related events). This risk persists until significant remission occurs. As improvement may not occur during the first few weeks or more of treatment, patients should be closely monitored until such improvement occurs. It is general the risk of suicide may increase in the early stages of recovery. Other psychiatric conditions for which venlafaxine is prescribed can also be associated with an increased risk of suicide related events. In addition, these conditions may be co-morbid with major depressive disorder. The same precautions observed when treating patients with major depressive disorder should therefore be observed when treating patients with other psychiatric disorders. Patients with a history of suicide-related events, or those exhibiting a significant degree of suicidal ideation prior to commencement of treatment, are known to be at greater risk of suicidal thoughts or suicide attempts, and should receive careful monitoring during treatment. A meta-analysis of antidepressant drugs in adult patients with psychiatric disorders showed an increased risk of suicidal behavior with antidepressants compared to in patients less than 25 years old. Close supervision of patients, and in particular those at high risk, should accompany drug therapy, especially in early treatment and following dose changes. Patients (and caregivers of patients) should be alerted about the need to monitor for any clinical worsening, suicidal behavior or thoughts and unusual changes in behavior, and to seek medical advice immediately if these symptoms present.

Use in children and adolescents under 18 years of age

Venlafaxine prolonged-release capsules should not be used in the treatment of children and adolescents under the age of 18 years. Suicide-related behaviors (suicide attempt and suicidal thoughts) and hostility (predominantly aggression, oppositional behavior and anger) were more frequently observed among children and adolescents treated with antidepressants compared to those not treated. If, a decision to treat is nevertheless taken, the patient should be carefully monitored for the appearance of suicidal symptoms. In addition, long-term safety data in children and adolescents concerning growth, maturation and cognitive and behavioral development are lacking.

Serotonin syndrome

As with other serotonergic agents, serotonin syndrome, a potentially life threatening condition may occur with venlafaxine treatment, particularly with concomitant use of other agents that may affect the serotonergic neurotransmitter system (including triptans, SSRIs, SNRIs, lithium, sibutramine, St John's Wort [Hypericum perforatum], fentanyl and its analogues, tramadol, dextromethorphan, tapentadol, pethidine, methadone and pentazocine), with medicinal agents that impair metabolism of serotonin such as MAOIs e.g. methylene blue, with serotonin precursors (such as tryptophan supplements) or with antipsychotics or other dopamine antagonists. Serotonin syndrome symptoms may include mental status changes (e.g., agitation, hallucinations, coma), autonomic instability (e.g., tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g., hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g., nausea, vomiting, diarrhoea). Serotonin syndrome in its most severe form, can resemble NMS, which includes hyperthermia, muscle rigidity, autonomic instability with possible rapid fluctuation of vital signs and mental status changes. If concomitant treatment with venlafaxine and other agents that may affect the serotonergic and/or dopaminergic neurotransmitter systems is clinically warranted, careful observation of the patient is advised, particularly during treatment initiation and dose increases. The concomitant use of venlafaxine with serotonin precursors (such as tryptophan supplements) is not recommended.

Narrow-angle glaucoma

Mydriasis may occur in association with venlafaxine. It is recommended that patients with raised intraocular pressure or patients at risk for acute narrow-angle glaucoma (angle-closure glaucoma) be closely monitored.

Blood pressure

Dose-related increases in blood pressure have been commonly reported with venlafaxine. All patients should be carefully screened for high blood pressure and pre-existing hypertension should be controlled before initiation of treatment. Blood pressure should be reviewed periodically, after initiation of treatment and after dose increases. Caution should be exercised in patients whose underlying conditions might be compromised by increases in blood pressure, e.g., those with impaired cardiac function.

Heart rate

Increases in heart rate can occur, particularly with higher doses. Caution should be exercised in patients whose underlying conditions might be compromised by increases in heart rate.

Cardiac disease and risk of arrhythmia

Venlafaxine has not been evaluated in patients with a recent history of myocardial infarction or unstable heart disease. Therefore, it should be used with caution in these patients. The balance of risks and benefits should be considered before prescribing venlafaxine to patients at high risk of serious cardiac arrhythmia.

Convulsions

Convulsions may occur with venlafaxine therapy. As with all antidepressants, venlafaxine should be introduced with caution in patients with a history of convulsions, and concerned patients should be closely monitored. Treatment should be discontinued in any patient who develops seizures.

Hyponatraemia

Cases of hyponatraemia and/or the Syndrome of Inappropriate Antidiuretic Hormone (SIADH) secretion may occur with venlafaxine. This has most frequently been reported in volume-depleted or dehydrated patients. Elderly patients, patients taking diuretics, and patients who are otherwise volume-depleted may be at greater risk for this event.

Abnormal bleeding

Medicinal products that inhibit serotonin uptake may lead to reduced platelet function. Bleeding events related to SSRI and SNRI use have ranged from ecchymoses, hematomas, epistaxis, and petechiae to gastrointestinal and life threatening haemorrhages. The risk of haemorrhage may be increased in patients taking venlafaxine. As with other serotonin-reuptake inhibitors, venlafaxine should be used cautiously in patients predisposed to bleeding, including patients on anticoagulants and platelet inhibitors.

Serum cholesterol

Measurement of serum cholesterol levels should be considered during long-term treatment.

Co-administration with weight loss agents

The safety and efficacy of venlafaxine therapy in combination with weight loss agents, including phentermine, have not been established. Co-administration of venlafaxine and weight loss agents is not recommended. Venlafaxine is not indicated for weight loss alone or in combination with other products.

Mania/hypomania

Mania/hypomania may occur in a small proportion of patients with mood disorders who have received antidepressants, including venlafaxine. As with other antidepressants, venlafaxine should be used cautiously in patients with a history or family history of bipolar disorder.

Aggression

Aggression may occur in a small number of patients who have received antidepressants, including venlafaxine. This has been reported under initiation, dose changes and discontinuation of treatment. As with other antidepressants, venlafaxine should be used cautiously in patients with a history of aggression.

Discontinuation of treatment

Withdrawal symptoms, when treatment is discontinued, are common, particularly if discontinuation is abrupt. The risk of withdrawal symptoms may be dependent on several factors, including the duration and dose of therapy and the rate of dose reduction. Dizziness, sensory disturbances (including paresthesia), sleep disturbances (including insomnia and intense dreams), agitation or anxiety, nausea and/or vomiting, tremor and headache are the most commonly reported reactions. Generally, these symptoms are mild to moderate; however, in some patients they may be severe in intensity. They usually occur within the first few days of discontinuing treatment, but there have been very rare reports of such symptoms in patients who have inadvertently missed a dose. Generally, these symptoms are self-limiting and usually resolve within 2 weeks, though in some individuals they may be prolonged (2-3 months or more). It is therefore advised that venlafaxine should be gradually tapered when discontinuing treatment over a period of several weeks or months, according to the patient's needs.

Akathisia/psychomotor restlessness

The use of venlafaxine has been associated with the development of akathisia, characterised by a subjectively unpleasant or distressing restlessness and need to move often accompanied by an inability to sit or stand still. This is most likely to occur within the first few weeks of treatment. In patients who develop these symptoms, increasing the dose may be detrimental.

Dry mouth

Dry mouth is reported in 10% of patients treated with venlafaxine. This may increase the risk of caries, and patients should be advised upon the importance of dental hygiene.

Diabetes

In patients with diabetes, treatment with an SSRI or venlafaxine may alter glycaemic control. Insulin and/or oral antidiabetic dosage may need to be adjusted.

Interactions with other medicaments

Monoamine Oxidase Inhibitors (MAOI)

Irreversible non-selective MAOIs

Venlafaxine must not be used in combination with irreversible non-selective MAOIs. Venlafaxine must not be initiated for at least 14 days after discontinuation of treatment with an irreversible non-selective MAOI. Venlafaxine must be discontinued for at least 7 days before starting treatment with an irreversible non-selective MAOI.

Reversible, selective MAO-A inhibitor (moclobemide)

Due to the risk of serotonin syndrome, the combination of venlafaxine with a reversible and selective MAOI, such as moclobemide, is not recommended. Following treatment with a reversible MAOI-inhibitor, a shorter withdrawal period than 14 days may be used before initiation of venlafaxine treatment. It is recommended that venlafaxine should be discontinued for at least 7 days before starting treatment with a reversible MAOI.

■ Black A/s: 210 x 480 mm

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		Binoven ER Capsules	Leaflet	P1540018	17.07.2025 & 04.40 pm
Team Leader Kiran		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
		Malaysia_U3	00	Submission	
Initiator	Sainath	Dimensions	No. of Colours : 01		
Artist:  <i>MJ Designers</i> Ashok		Pharmacode	 40018		
		40018			
Additional Information :					

Reversible, non-selective MAOI (linezolid)

The antibiotic linezolid is a weak reversible and non-selective MAOI and should not be given to patients treated with Venlafaxine. Severe adverse reactions have been reported in patients who have recently been discontinued from an MAOI and started on venlafaxine, or have recently had venlafaxine therapy discontinued prior to initiation of an MAOI. These reactions have included tremor, myoclonus, diaphoresis, nausea, vomiting, flushing, dizziness, and hyperthermia with features resembling neuroleptic malignant syndrome, seizures, and death.

Serotonin syndrome

As with other serotonergic agents, serotonin syndrome, a potentially life-threatening condition, may occur with venlafaxine treatment, particularly with concomitant use of other agents that may affect the serotonergic neurotransmitter system (including triptans, SSRIs, SNRIs, lithium, sibutramine, St. John's Wort [Hypericum perforatum], fentanyl and its analogues, tramadol, dextromethorphan, tapentadol, pethidine, methadone and pentazocine), with medicinal agents that impair metabolism of serotonin (such as MAOIs e.g. methylene blue), with serotonin precursors (such as tryptophan supplements) or with antipsychotics or other dopamine antagonists. If concomitant treatment with venlafaxine and an SSRI, an SNRI or a serotonin receptor agonist (triptan) is warranted, careful observation of the patient is advised, particularly during treatment initiation and dose increases. The concomitant use of venlafaxine with serotonin precursors (such as tryptophan supplements) is not recommended.

CNS-active substances

The risk of using venlafaxine in combination with other CNS-active substances has not been systematically evaluated. Consequently, caution is advised when venlafaxine is taken in combination with other CNS-active substances.

Ethanol

Venlafaxine has been shown not to increase the impairment of mental and motor skills caused by ethanol. However, as with all CNS-active substances, patients should be advised to avoid alcohol consumption.

Effect of other medicinal products on Venlafaxine

Concomitant use of CYP3A4 inhibitors (e.g., atazanavir, clarithromycin, indinavir, itraconazole, voriconazole, posaconazole, ketoconazole, nefinavir, ritonavir, saquinavir, telithromycin) and venlafaxine may increase levels of venlafaxine and O-desmethylvenlafaxine. Therefore, caution is advised if a patient's therapy includes a CYP3A4 inhibitor and Venlafaxine concomitantly.

Effect of venlafaxine on other medicinal products

Lithium

Serotonin syndrome may occur with the concomitant use of venlafaxine and lithium (see Serotonin syndrome).

Diazepam

Venlafaxine has no effects on the pharmacokinetics and pharmacodynamics of diazepam and its active metabolite, desmethyldiazepam. Diazepam does not appear to affect the pharmacokinetics of either venlafaxine or O-desmethylvenlafaxine. It is unknown whether a pharmacokinetic and/or pharmacodynamics interaction with other benzodiazepines exists.

Imipramine

Venlafaxine did not affect the pharmacokinetics of imipramine and 2-OH-imipramine. There was a dose dependent increase of 2-OH-desipramine AUC by 2.5 to 4.5-fold when venlafaxine 75 mg to 150 mg daily was administered. Imipramine did not affect the pharmacokinetics of venlafaxine and O-desmethylvenlafaxine. The clinical significance of this interaction is unknown. Caution should be exercised with co-administration of venlafaxine and imipramine.

Haloperidol

Haloperidol has shown a 42% decrease in total oral clearance, a 70% increase in AUC, an 88% increase in C_{max}, but no change in half-life for haloperidol. This should be taken into account in patients treated with haloperidol and venlafaxine concomitantly. The clinical significance of this interaction is unknown.

Risperidone

Venlafaxine increased the risperidone AUC by 50%, but did not significantly alter the pharmacokinetic profile of the total active moiety (risperidone plus 9- hydroxyrisperidone). The clinical significance of this interaction is unknown.

Metoprolol

Metoprolol did not alter the pharmacokinetic profile of venlafaxine or its active metabolite, O-desmethylvenlafaxine. Caution should be exercised with co-administration of venlafaxine and metoprolol.

Indinavir

Indinavir did not affect the pharmacokinetics of venlafaxine and O- desmethylvenlafaxine.

Pregnancy and Lactation

Pregnancy

There are no adequate data from the use of venlafaxine in pregnant women. The potential risk for humans is unknown. Venlafaxine must only be administered to pregnant women if the expected benefits outweigh any possible risk. As with other serotonin reuptake inhibitors (SSRIs/SNRIs), discontinuation symptoms may occur in the newborns if venlafaxine is used until or shortly before birth. Some newborns exposed to venlafaxine late in the third trimester have developed complications requiring tube-feeding, respiratory support or prolonged hospitalization. Such complications can arise immediately upon delivery. The use of SSRIs in pregnancy, particularly in late pregnancy, may increase the risk of persistent pulmonary hypertension in the newborn (PPHN). Although no studies have investigated an association of PPHN to SNRI treatment, this potential risk cannot be ruled out with venlafaxine taking into account the related mechanism of action (inhibition of the re-uptake of serotonin). The following symptoms may be observed in neonates if the mother has used an SSRI/SNRI late in pregnancy: irritability, tremor, hypotonia, persistent crying, and difficulty in sucking or in sleeping. These symptoms may be due to either serotonergic effects or exposure symptoms. In the majority of cases, these complications are observed immediately or within 24 hours after partus.

Lactation

Venlafaxine and its active metabolite, O-desmethylvenlafaxine, are excreted in breast milk. Symptoms consistent with venlafaxine drug discontinuation have also been reported after stopping breast-feeding. A risk to the suckling child cannot be excluded. Therefore, a decision to continue/discontinue breastfeeding or to continue/discontinue therapy with Venlafaxine should be made, taking into account the benefit of breast-feeding to the child and the benefit of Venlafaxine therapy to the woman.

Effects on ability to drive and use machines

Any psychoactive medicinal product may impair judgment, thinking, and motor skills. Therefore, any patient receiving venlafaxine should be cautioned about their ability to drive or operate hazardous machinery.

Side Effects / Adverse Effects / Undesirable Effects

Body System

Blood and lymphatic system disorders

Rare: Agranulocytosis, Aplastic anaemia, Pancytopenia, Neutropaenia

Very Rare: Thrombocytopaenia

Immune system disorders

Rare: Anaphylactic reaction

Endocrine disorders

Rare: Inappropriate antidiuretic hormone secretion

Very Rare: Blood prolactin increased

Metabolism and nutrition disorders

Common: Decreased appetite

Rare: Hyponatraemia

Psychiatric disorders

Very Common: Insomnia

Common: Confusional state, Depersonalization, Abnormal dreams, Nervousness, Libido decreased, Agitation, Anorgasmia,

Uncommon: Mania, Hypomania, Hallucination, Derealization, Abnormal orgasm, Bruxism, Apathy

Rare: Delirium

Not Known: Suicidal ideation and suicidal behaviours, Aggression

Nervous system disorders

Very Common: Headache, Dizziness and Sedation

Common: Akathisia, Tremor, Paraesthesia, Dysgeusia

Uncommon: Syncope, Myoclonus, Balance disorder, Coordination abnormal, Dyskinesia

Rare: Neuroleptic Malignant Syndrome (NMS), Serotonin syndrome, Convulsion, Dystonia

Very Rare: Tardive dyskinesia

Eye disorders

Common: Visual impairment, Accommodation disorder, including vision blurred, Mydriasis

Rare: Angle-closure glaucoma

Ear and labyrinth disorders

Common: Tinnitus

Not Known: Vertigo

Cardiac disorders

Common: Tachycardia, Palpitations

Rare: Torsade de pointes, Ventricular tachycardia, Ventricular fibrillation, Electrocardiogram QT prolonged

Vascular disorders

Common: Hypertension, Hot flush

Uncommon: Orthostatic hypotension, Hypoension

Respiratory, thoracic and mediastinal disorders

Common: Dyspnoea, Yawning

Rare: Interstitial lung disease, pulmonary eosinophilia

Gastrointestinal disorders

Very Common: Nausea, Dry mouth, Constipation

Common: Diarrhoea, Vomiting

Uncommon: Gastrointestinal haemorrhage

Rare: Pancreatitis

Hepatobiliary disorders

Uncommon: Liver function test abnormal

Rare: Hepatitis

Skin and subcutaneous tissue disorders

Very Common: Hyperhidrosis (including night sweats) Common: Rash, Pruritus

Uncommon: Urticaria, Alopecia, Ecchymosis, Angioedema, Photosensitivity reaction

Rare: Stevens-Johnson syndrome, Toxic epidermal necrolysis, Erythema multiforme

Musculoskeletal and connective tissue disorders

Common: Hypertonia

Rare: Rhabdomyolysis

Renal and urinary

Common: Urinary hesitation, Urinary retention, Pollakiuria Uncommon: Urinary incontinence

Reproductive system and breast disorders

Common: Menorrhagia, Metrorrhagia, Erectile dysfunction, Ejaculation disorder

General disorders and administration site conditions

Common: Fatigue, Asthenia, Chills

Very Rare: Mucosal haemorrhage

Overall

Common: Weight decreased, Weight increased, Blood cholesterol increased

Very Rare: Bleeding time prolonged

Discontinuation of treatment

Discontinuation of venlafaxine (particularly when abrupt) commonly leads to withdrawal symptoms. Dizziness, sensory disturbances (including paraesthesia), sleep disturbances (including insomnia and intense dreams), agitation or anxiety, nausea and/or vomiting, tremor, vertigo, headache and flu syndrome are the most commonly reported reactions. Generally, these events are mild to moderate and are self-limiting; however, in some patients, they may be severe and/or prolonged. It is therefore advised that when venlafaxine treatment is no longer required, gradual discontinuation by dose tapering should be carried out.

Paediatric population

As with adults, decreased appetite, weight loss, increased blood pressure, and increased serum cholesterol were observed. In Paediatric the adverse reaction suicidal ideation was observed. There were also increased hostility and, especially in major depressive disorder, self-harm. Particularly, the following adverse reactions were observed in Paediatric patients: abdominal pain, agitation, dyspepsia, ecchymosis, epistaxis, and myalgia.

Symptoms and Treatment of Overdose

Overdose with venlafaxine was reported predominantly in combination with alcohol and/or other medicinal products. The most commonly reported events in overdose include tachycardia, changes in level of consciousness (ranging from somnolence to coma), mydriasis, convulsion, and vomiting. Other reported events include electrocardiographic changes (e.g., prolongation of QT interval, bundle branch block, QRS prolongation), ventricular tachycardia, bradycardia, hypotension, vertigo, and death. Prescriptions for venlafaxine should be written for the smallest quantity of the medicinal product consistent with good patient management in order to reduce the risk of overdose.

Recommended treatment

General supportive and symptomatic measures are recommended; cardiac rhythm and vital signs must be monitored. When there is a risk of aspiration, induction of emesis is not recommended. Gastric lavage may be indicated if performed soon after ingestion or in symptomatic patients. Administration of activated charcoal may also limit absorption of the active substance. Forced diuresis, dialysis, hemoperfusion and exchange transfusion are unlikely to be of benefit. No specific antidotes for venlafaxine are known.

Incompatibilities

Not applicable

Storage condition

Store at below 30°C.

Shelf Life

Venlafaxine Extended Release Capsules 37.5 mg, 75 mg & 150 mg: 36 months

Presentation

Extended-release cap 37.5 mg x 30's. 75 mg x 30's.

150 mg x 30's.

Manufactured by


AUROBINDO
Aurobindo Pharma Limited,
Unit III Survey No. 313 314, Bachupally Village, Bachupally Mandal,
Medchal Malkajgiri District, Hyderabad, 500090, India.

Product Registration Holder in Malaysia:

UNIMED SDN BHD

No. 53, Jalan Tembaga SD 5/2B,

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